

Dental Tool Kit



*A guide for caregivers of autistic children
and their dental professionals*

My Autism Guide
SIGN UP



Information
based on
your needs

guide.autismspeaks.org

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Introduction

Oral health is an important healthcare need. But proper dental hygiene can be challenging for children and adolescents with autism spectrum disorder (ASD). Some may have sensitivities to brushing, flossing, rinsing or having any dental equipment in their mouth. Sensory needs may result in some biting their nails, grinding their teeth, chewing on items like pencils or pens and putting other non-food items in their mouth that can adversely affect their teeth and gums. Some may have difficulty communicating mouth pain and/or worry about common occurrences like a loose baby tooth or food stuck between teeth. All of these can result in a delay of or improper dental care and ultimately poor dental health.

This guide is designed to help you develop healthy dental hygiene habits with your autistic loved one so they can enjoy a lifetime of proper oral care.

Acknowledgments

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For caregivers

Getting started at home

Good dental hygiene habits for a child start at infancy, according to the [American Academy of Pediatrics](#). Parents and caregivers should wipe their gums with a soft cloth after each feeding. When the child's first tooth pops through, it is time to use a small, soft toothbrush designed for infants. Establishing a dental routine like this at a set time every day, and eventually twice a day, is one of the most important things you can do for your child.

Brushing

We use a toothbrush that is connected to an app – so it's a game – making sure each area is adequately brushed for long enough. We have also used toothbrushes that have lights or vibrating prompts when it is time to switch areas.

— **Lindsay Austin, Autism Speaks Legal, Risk and Compliance Director**

Starting at age 2, children can start learning to brush their own teeth and about the importance of doing it twice a day. But brushing can be difficult for autistic children. Among the issues they can experience: sensory issues with a toothbrush or with the texture or taste of toothpaste, difficulty with fine motor skills, behaviors that require extra care and attention during brushing, anxiety or general fear of what might happen or just a general aversion to doing it.

But with patience, practice and maybe even some prizes, your child can get used to brushing and even learn to independently brush regularly. The following are tips from dental professionals, medical providers, autistic adults and parents of autistic children to help you and your child get to that point.

- Let your child pick out their toothbrush if they are willing and able – it is important that the brush is the right size for your child's mouth and that it has soft bristles
 - Your dentist can recommend the best size and give you samples before you buy one
 - Major brands now offer several options in a variety of colors, with kid-friendly characters and even with music, lights and built-in timers that control both
 - It is best to start with a manual toothbrush, but powered toothbrushes are safe if the child can handle using them, tolerate the noise and is comfortable with the vibration

- If your child has an aversion to a standard toothbrush, there are alternatives on the market, advertised as no-fuss, sensory-friendly and even autism-friendly – just make sure they have the [American Dental Association \(ADA\) Seal](#) – your assurance that the product has been objectively evaluated for safety and efficacy by the ADA Council on Scientific Affairs
- Let your child pick out a toothbrush holder and pick out a safe, clean spot for it
- Let your child pick out a toothpaste that contains the age-appropriate amount of fluoride – like with toothbrushes, many brands have character-themed tubes
- You may want to start by using the toothbrush to touch your child's lips or just inside the mouth – you may also want to teach your child to "open wide," so that this direction is understood
- Show-and-tell: have your child watch you while you brush your teeth and discuss each step as you do it
- Create a visual chart outlining the steps to brushing teeth and hang it near the bathroom sink or use erasable markers and write the steps on the bathroom mirror

Ten steps to brushing

1. Put toothpaste on the toothbrush. (The ADA recommends a rice-sized smear for children ages 0 to 3 years and a pea-size amount for children ages three and up.)
2. Brush the bottom back teeth first. Start on the right side. Gently move the toothbrush back and forth on the outside, inside and top (chewing surface) of the bottom back teeth five times. Repeat on the left side. Pause and spit out any toothpaste into the sink, if needed.
3. Move up and brush the top back teeth. Start on the right side. Gently move the toothbrush back and forth on the inside, outside and bottom (chewing surfaces) five times. Repeat on the left side. Pause and spit out any toothpaste into the sink, if needed.
4. Brush the bottom front teeth outside and inside five times. Pause and spit out any toothpaste into the sink, if needed.
5. Move to the top front teeth and brush the outside and inside five times. Pause and spit out any toothpaste into the sink, if needed.
6. Brush the top front teeth inside, outside, and chewing surfaces five times. Pause and spit out any toothpaste into the sink, if needed.
7. Brush the tongue gently.
8. Rinse the mouth with water and spit the toothpaste into the sink.
9. Rinse the toothbrush and put it back in the toothbrush holder.
10. Put the cap on the toothpaste.

Flossing

Another important part of oral health is flossing. Just like brushing, this should be introduced early, as soon as a child has two teeth touching.

Like brushing, flossing can prevent some issues with autistic children. To help them, instead of using your typical 18-inch-long piece of floss wrapped around your fingers, consider alternatives like handheld flossers. Like toothbrushes and toothpaste, they come in different colors and with character themes and typically are ergonomically friendly making them easy to hold. Or even use a water flosser if your child can handle the gentle stream of pulsating water on their gums. You should also do a show-and-tell with them using whichever method they can handle and be prepared to help them as needed.

Do not worry about flossing before or after brushing. Both are acceptable so let your child pick which they prefer. The most important thing is to make sure they floss between each tooth. Unlike brushing, you only need to floss once a day. So either let your child pick whether they prefer to do it morning or night, or pick which time works best with their mood, and then make it part of your child's routine.

Other tips

- Get a timer and set it for two minutes – for many autistic children it is helpful to see their progress and be able to countdown to being done
- Put a clock in the bathroom with an alarm that is set to ring for the time in the morning and the evening when the child should brush their teeth

If your child struggles to hold a standard manual toothbrush, try placing a tennis ball or foam grip on the handle to make it easy to hold.

Keep a daily progress chart each week and reward your child when they successfully fill it out. Maybe you give them a favorite candy, drink or other treat that is on the list of sugary treats the dentist says to only have on special

occasions. Or take a cue from the dentist office: have a grab bag of trinkets and let them pick from it.

Preparing for the dentist

Autistic people may be prone to poor dental health because of dental anxiety. Children with ASD commonly exhibit uncooperative behaviors during dentist visits, which impedes their oral care and may be tied to anxiety about dental visits and overall oral hygiene.

Finding the right dentist

It is important to find a dentist that works successfully with individuals with autism or is willing to work with them. Your pediatrician or other providers who support your child should be able to recommend one. Or you can look for one in your local area in the [Autism Speaks Resource Guide](#). If you are unable to find a dentist with enough experience treating patients with autism, call and ask if the dentist is open to seeing your child. A good rule of thumb is: if the dentist says yes, then they are willing to work with you and your child to meet your needs.

Once you have the name of a dentist or a dental practice, call and see if you can make a pre-visit appointment with your child. Ask for a tour of the office that includes a visit to the dentist's chair so your child can get comfortable with the environment.

You should also speak with the dentist and dental hygienist and let them know:

- What time of day works best for your child
- Concerns your child has about the dentist and what happens during a visit
- Sensory issues your child has
- Challenges that may present during the visit (For instance, tell them if your child typically has problems "opening wide" when brushing their teeth or if they bite on their toothbrush)

You may want to find out if the dentist has a Standard Dental Form that you can fill out before your appointment. Our example of the [Standard Dental Form](#) is also provided in the **Appendix** of this guide.

Preparing for the exam

Preparation is key to a successful dental visit. One way to prepare is with a [Visual Schedule](#) of what will happen. Our example of the Visual Schedule is also provided in the **Appendix** of this guide. Giving your child the opportunity to see the step-by-step guide of an exam can give them time to process what they will experience and provide them opportunities to ask questions about each step, which can relieve some anxiety.

A visual exam can also serve as a practice guide. For instance, you can have your child practice sitting in a reclining chair and then trying each of the following steps:

- Putting their hands on their stomach and relaxing
- Putting their feet out straight
- Opening wide
- Holding their mouth open
- Counting their teeth

You can also purchase many of the instruments used at a dental visit at a drugstore and then show your child how they work like:

- Dental mirror
- Water flosser
- Rubber-tipped gum massager

You can even ask your dentist in advance of the visit if you could have a set of dental bite wings so that your child may practice biting down on them when they need to have x-rays.

Consistency is key. Commit to taking your child every six months so they get used to the experience. And your provider may be open to having you visit more frequently for a mock appointment.

— Brooke Horowitz, Autism Speaks Research and Prospect Development

Meeting the dentist and the staff

You may want to schedule a pre-visit consultation the first time you go to the dentist or switch to a new dentist. This would be a good time to share the [Standard Dental Form](#) and supply any other experiences you think would help the dentist and hygienist better understand your child. For example, you should inform the dentist of any signs of distress your child displays. While there you should also:

- See if it is possible for your child to see a typical exam room and chair, where x-rays are done and just look around and see the lights and hear the sounds
- Ask about being present with your child during their exam, in front of your child – except for some cases involving x-rays, both the hygienist and the dentist should be comfortable with you being there

- Talk about how long a typical appointment lasts so your child knows what to expect – also ask about breaks between or check-ins with your child. If necessary, ask to book a longer appointment
- Make sure it is ok for your child to have a favorite fidget, stuffed animal or toy to hold, or in sight if it is too big

Make the appointment for either first thing in the morning or the first appointment after lunch to limit wait times. Having low wait times is key for a successful dental visit for my son.

— **J-Jaye Hurley, Autism Speaks Autism Response Team**

Before you leave for your appointments, you may want to call ahead to see if the dentist is running on time. If they are delayed and you think that your child may be anxious in the waiting area, you may want to ask the receptionist if you could wait in the car and ask them to call you on your cell phone when the dentist is ready. You may also want to bring a family member or support person to help make the visit a success.

Most pediatric dentist offices give a goody bag or prize out after each visit. But you may also want to have a reward ready for your child for a job well done and to encourage them to go again for their next check-up.

Special consideration: sedation and protective stabilization

With patience and understanding, many children with autism can sit through a normal dental visit.

For some, all they need is experience getting used to going to the dentist. For others, breaking appointments up into shorter visits helps. For example, maybe X-rays are done during one visit and cleaning is done in another. But for those for whom the dentist is too much, protective stabilization and sedation may need to be considered.

Protective stabilization

Protective stabilization is an option when the dental patient can't be persuaded to sit still in the dental chair, according to Dr. Elizabeth Shick, DDS, MPH, clinical associate professor at the University of Colorado's School of Dental Medicine and co-author of [Autism Speaks ATN/AIR-P Tool Kit for Dental Professionals](#).

It is one way to ensure safety for both the patient and the dental staff. This can be as simple as having a parent, caregiver or dental assistant hold the patient's hands. If this isn't enough, a wrapping device made from soft cloth may be used to help stabilize the patient's arms and legs.

It is perfectly appropriate for you to ask the staff if they've received training in protective stabilization, sometimes called "medical immobilization." Many dentists feel comfortable with medical immobilization because it's the safest way to perform dental procedures when the patient can't be depended on to hold still.

But it is also important to know that protective stabilization is not right for every autistic person or their parent. You and your child should always be asked first if it is ok to try protective stabilization. If you do try it and your child is in distress with it or you are uncomfortable, it is ok to remove it and discuss alternatives with the dental staff. The goal should always be to build trust with the dentist, not increase anxiety or introduce apprehension.

...I would like us to find a dental practice in the same office park as [my daughter's] ABA center and see if they would let the BCBA bring her in for short times to practice. She's recently gotten used to using a timer on an iPhone and we used it successfully to practice her laying still for an EEG. It worked great! Now granted, an EEG is not invasive like a teeth cleaning is but I would like to see if we can use the experience to develop something similar for basic dental procedures. Another idea is to get a portable iPad floor stand so that we could pair those practice sessions with one of her favorite calming videos.

— **Christa Stevens, JD, MAT, Autism Speaks State Government Affairs Director**

Sedation

It is important to note that before dental professionals even offer or administer sedation, most states require that they complete advanced training and licensing. So be sure to ask about your dentist's qualifications and credentials and ask what precautions they have in place. For instance, your child's vitals should be monitored the whole time no matter the sedation method used. It is also recommended you first bring your child to their pediatrician for a medical exam and discuss both sedation and alternatives.

Nitrous oxide. Inhaled through a nose mask, nitrous oxide is a generally safe anti-anxiety medicine. It leads to a state of euphoria, explaining its nickname, 'laughing gas.' The patient must continue breathing through the nose until the dental procedure is complete. So it's not a good option for children younger than four or five or anyone who is crying or otherwise

breathing through the mouth. In addition, a small minority of patients simply don't respond to nitrous oxide. For some, it causes nausea, though this rarely lasts for more than a few hours after the appointment.

Conscious sedation. Conscious sedation involves the use of sedatives to produce a calm, sleepy state without loss of consciousness. Individuals with autism vary widely in how they respond to conscious sedation. Sedatives can have side effects, too. This is why it is a good idea to take your child to the pediatrician when considering sedation. The dentist should also carefully screen your child. They need to rule out respiratory problems, evaluate tonsil size and look for other medical contraindications to make sure your child is a good candidate.

Additionally, during conscious sedation, an appropriately trained assistant should be with your child the whole time and must carefully and continually monitor their vitals.

Typically, the sedative is given as a pill or liquid one hour before the procedure. The most common choices include midazolam (Versed), hydroxyzine pamoate (Vistaril), meperidine (Demerol) or chloral hydrate. Sometimes the dentist may want to use stabilization restraints and supplemental oxygen during the procedure.

General anesthesia. General anesthesia, which involves loss of consciousness, is a last choice for those who don't respond to the above options. It must be administered by a dental or medical anesthesiologist or certified registered nurse anesthetist in a hospital or fully equipped healthcare center. General anesthesia brings a number of health risks. Generally, it's not recommended for dental cleanings or other routine dental procedures.

Additionally, there are likely financial considerations when opting for general anesthesia. It is generally not covered by dental insurance, or if it is, will likely be more expensive than the maximum yearly benefit under most dental plans. This means that parents will be responsible for the balance. It is also not uncommon for the dental office to require full payment for sedation up front. If sedation is going to be needed for every visit, sedation will be a significant yearly expense. In these cases, parents may want to prioritize making the goal of dental care without sedation in a child's applied behavior analysis (ABA) treatment plan. Consultation with a Board Certified Behavior Analyst (BCBA) could be a helpful step toward this.

When our son was eligible the local children's hospital worked out very well as he needed to be put to sleep for major procedures including cavities and they have a lot of experience with special needs.

— Jeff Rickel, Autism Speaks Database Manager

Autistic perspective: Coping at the dentist

by Sean Sullivan, IKnowAutism.org Foundation, Executive Director & Founder

Every part of going to the dentist is stressful except the end. The most stressful part though is the tension that builds. When I go to the dentist, or anyone that goes, not just me, I worry there is something wrong and about what is going to happen to my teeth. I don't want to be there at all.

After trying many coping mechanisms throughout my life, it finally got easier to go the dentist when I was in my twenties. So I want to share some of what I learned to help other autistic people.

For the dentist

It would be nice if you could provide noise canceling headphones or allow them. Almost every noise in the dentist office is unfamiliar, unpleasant and sometimes scary. The sounds can hurt more than anything you do in the mouth. Headphones would help and then we could work on hand signals so we can communicate when you need me to take them off.

Every dentist should give out squish balls ahead of the visit. They are a good stress reliever and something to focus on instead of what you are doing.

For dental staff and other patients at the dentist

If you see someone who might be walking back and forth, flapping their hands or putting their hands over their ears which are common for people with autism, please keep in mind that they are not doing those behaviors to try to scare people and that those are not scary behaviors. They are doing those behaviors as a coping mechanism for different things that are external in their environment.

For fellow autistic people who do not like the dentist

Learn breathing techniques so you can do them during the appointments.

Practice meditation. I do some meditation before I go but you can meditate any time during the visit.

Put it into perspective. One thing I would say would be to keep in mind that you only go to the dentist once every 6 months and it's only for a half an hour to an hour. So in a year, that is only one to two hours of your whole year.

Reward yourself for getting through your appointment and scheduling your next one.

Sean is a self-advocate and a member of California's Statewide Self Advocacy Network, part of the State Council on Developmental Disabilities as the Orange County representative. His mission is to help other people with autism and to inspire people.

For dental professionals

What is autism?

Autism, or autism spectrum disorder (ASD), refers to a broad range of conditions characterized by challenges with social skills, repetitive behaviors, speech and nonverbal communication. According to the Centers for Disease Control, autism affects an estimated 1 in 36 children and 1 in 45 adults in the United States today.

Autism looks different for everyone, and each person with autism has a distinct set of strengths and challenges. Some autistic people can speak, while others are nonverbal or minimally verbal and communicate in other ways. Some have intellectual disabilities, while some do not. Some require significant support in their daily lives, while others need less support and, in some cases, live entirely independently.

On average, autism is diagnosed around age 5 in the U.S., with signs appearing by age 2 or 3. Current diagnostic guidelines in the DSM-5-TR break down the ASD diagnosis into three levels based on the amount of support a person might need: level 1, level 2, and level 3. See more information about each level. Anybody can be autistic, regardless of sex, age, race or ethnicity. However, research from the CDC says that boys get diagnosed with autism four times more often than girls.

Many people with autism experience other medical, behavioral or mental health issues that affect their quality of life – and could affect their dental health and visits. Among the most common co-occurring conditions are:

- attention-deficit/hyperactivity disorder (ADHD)
- anxiety and depression
- gastrointestinal (GI) disorders
- seizures and sleep disorders

Children and adults with autism may have difficulty in these areas:

Social interactions

- Starting and taking turns in conversations
- Sharing interests or emotions
- Understanding what others are thinking or feeling

Communication

- Making eye contact
- Understanding other people's body language, gestures and facial expressions
- Regulating tone of voice (e.g. they may speak too loudly, too quietly and/or with a monotone voice)

Developing, maintaining and understanding relationships

- Expressing feelings and seeking emotional comfort from others
- Making friends and playing with peers
- Understanding boundaries and personal space
- Feeling overwhelmed in social situations

Repetitive movements, play or speech patterns

- Stimming, or making repetitive body movements to regulate emotions (e.g. rocking, hand flapping, spinning, running back and forth)
- Lining up toys in a row, spinning wheels, repeatedly flipping switches
- Imitating another person's speech, repeating words or phrases (also known as echolalia)

Insistence on sameness and need for routine

- Extreme distress at even small changes in plans or routine
- Ritualistic behaviors (e.g. watching the same videos over and over, repeatedly touching objects in a set order)
- Need for routine (e.g. same daily schedule, meal menu, clothes, route to school)

Intense and highly focused interests

- Extreme interest or knowledge of specific, narrow topics
- Strong attachment to a certain object (e.g. a toy or figurine)

Under- or over-sensitivity to sensory stimulation

- Sensory differences, like unusual sensitivity to light, sound, touch or texture

- Lack of sensitivity to pain or temperature
- Sensory-seeking behaviors (e.g. smelling or touching objects, visual fascination with lights or movement)

Other characteristics of autism:

- Using other kinds of communication besides spoken language (e.g. typing on a computer, pointing to pictures on a tablet, or communicating through behavior)
- Difficulty with executive functioning (e.g. planning how to complete a task, juggling multiple tasks, making decisions)
- Trouble with fine motor skills and coordination
- Needing help with daily living skills
- Difficulty regulating and/or communicating emotions, sometimes resulting in harmful or self-injurious behaviors, sensory overload, meltdowns or shutdowns

Getting to know your autistic patient

In addition to having each parent of an autistic child fill out a form like the [Standard Dental Form](#) in the Appendix, it is recommended that you offer a pre-visit consultation with the parent and autistic child to allow time to get to know them and them to get to know you and your office, too. While every child is unique, some basic tips that are generally helpful in treating children with autism are:

- Use a calm voice
- Speak in plain terms, do not try to make jokes or use puns
- Give the child time to ask questions

During appointments and procedures, it will be important to offer the autistic child breaks and a chance to ask questions.

Appointments are child-led – the goal is for him to have a positive experience...We get done what can be done and whatever we can't get done waits. If he starts to get stressed, we stop until he is ready to continue, or we end the appointment...if the x-rays aren't perfect, we try again next appointment. Everyone uses tons of praise and positive affirmations to keep the appointment really positive.

— **Lindsay Austin, Autism Speaks Legal, Risk and Compliance Director**

Office accommodations to consider

Many autistic people have a difficult time dealing with the dental environment. The sights, sounds and smells of the office can be overwhelming, and the dental exam is intrusive.

Making physical accommodations to your office space can be a difference maker in these children having a successful visit and in turn, long-term positive dental health. Among them:

- Dim the lights if necessary
- Turn down loud noises
- Turn on instruments so that the child can see them before the instruments go in their mouths
- Remove the clutter in your office that may distract the child or make them anxious
- Let the child know what you will be doing – you may want to show the child on their hand how you will be counting their teeth so that they know what is going to happen

- Make sure to provide clear and accurate information when speaking to the child
- End each visit on a positive note, so that you and your patient can build upon your success

Sometimes it may take several visits in order to complete a dental exam. If you work with the family on this process, you will build a relationship together that will result in a lifetime of good dental health for children with autism.

Tips for reducing the stimuli in the office for autistic patients

by Dr. Louis Siegelman, DDS, Dentalphobia.com

- Place a warm, cozy blanket over the patient
- Have the patient wear headphones with their favorite music
- Have them wear darkened glasses like sunglasses during their treatment
- Create a peaceful environment in which the doctor and team maintain a calm, soothing tone and volume when speaking to the patient and to one another
- Use a quiet electric handpiece when conducting dental treatment
- Distraction. Some patients find dental visits more tolerable when they can watch their favorite show or hold an object in their hands. Fidget toys can be helpful as long as the patient can sit still while moving their hands
- Positive verbal reinforcement reminds patients that they are doing well and are in a safe, friendly environment

Dr. Siegelman is a board-certified dentist and dentist anesthesiologist who has provided care to many patients on the autism spectrum in his New York City office. His website: www.dentalphobia.com



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DENTAL TOOL KIT

Appendix

A Visual Schedule for a Trip to the Dentist



We will be visiting the Dentist today to make sure my teeth and mouth are nice, clean, and healthy.



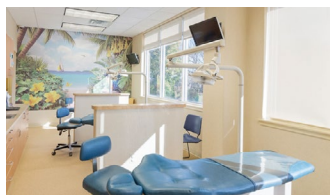
We'll walk into the office and tell our names to the people at the front desk.



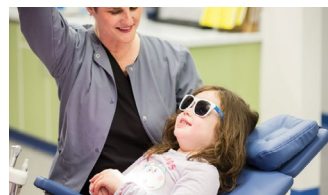
Until they call my name, we will play games, color, or watch television.



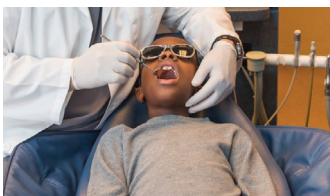
The hygienist will call my name and walk with me to a room with lots of special chairs.



The special chair will move up and down and lean back so the Dentist can look in my mouth.



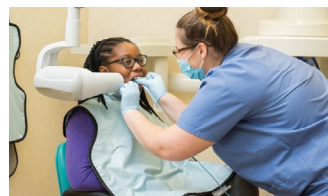
When I sit in the chair, I'll keep my hands on my belly and put my legs and feet straight out in front of me.



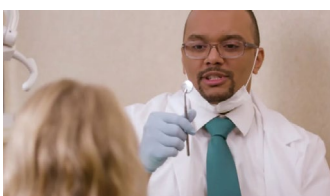
I'll open my mouth as wide as I can so the Dentist can look inside. There will be a bright light so they can see. I can ask for special sunglasses or close my eyes if it is too bright.



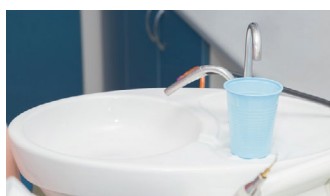
The Dentist will count my teeth with a special mirror. It may tickle a little.



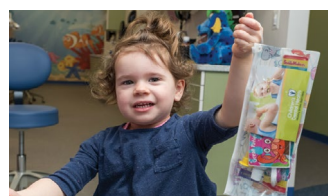
The Dentist will take special pictures of my teeth called X-Rays. I may have to sit down in a different chair and wear a heavy apron to take these pictures.



The Dentist will clean my teeth with special tools. They will show me each one and tell me what they are for before we use them.



When all my teeth are clean I will spit into the sink and rinse with water.



The Dentist will give me a special bag with a new toothbrush and toothpaste. I'll also get a special prize when I leave the office.



Standard dental form

MEDICAL INFORMATION

Patient Name: _____

Parent/Guardian: _____ Phone Number: _____

Parent/Guardian: _____ Phone Number: _____

Describe the nature of your child's disability:

Are they currently taking any medications? YES NO

If yes, what medications:

Has your child ever had seizures? YES NO If yes, date of last seizure: _____

Describe the type of seizure:

Does your child have any allergies? YES NO

If yes, please list:

Does your child wear a hearing aid? YES NO

If yes, please explain:

Does your child have any other physical challenges that the dental team should be aware of?



ORAL CARE

Has your child visited the dentist before? YES NO

If yes, please describe:

Please describe your child's at-home dental care:

My child uses: Powered toothbrush Manual toothbrush

Does your child floss? YES NO

My child brushes: Independently With parent/guardian's assistance

What are your dental health goals?

How often does your child snack during the day and on what types of foods?

COMMUNICATION & BEHAVIOR

Is your child able to communicate verbally? YES NO

Are there any useful phrases or words that work best with your child?

Are there certain cues that might help the dental team?

Does your child use non-verbal communication? YES NO

Please check any of the following that your child uses:

Mayer Johnson Symbols

Sign Language

Picture Exchange Communication System (PECS)

Sentence Board or Gestures

Communication app



COMMUNICATION AND BEHAVIOR

Will you be bringing a communication system with you? YES NO

If yes, please describe:

Are there any symbols/signs that the dental team can have available to assist with communication?

Are there any specific behavioral challenges that you would like the dental team to be aware of?

SENSORY ISSUES

Are there any sounds that your child is very sensitive to?

Does your child prefer the quiet? YES NO

Is your child more comfortable in a dimly lit room? YES NO

Is your child sensitive to motion and moving (i.e., the dental chair moving up and down or to a reclining position)?

Does your child have any specific oral sensitivities (gagging, gum sensitivities, etc.)?

Do certain tastes bother your child?

Is your child more comfortable in a clutter-free environment? YES NO

Please provide us with any additional information that may help us to prepare for a successful dental experience: